How GH Secretagogues Transformed My Training, My Recovery, My Energy and My Body at 70

We all feel the slow drift of aging: recovery takes longer, strength feels less reliable, fat settles around the waist, and sleep becomes lighter and more fragmented. Even with good nutrition, heavy lifting, and consistent sleep, I felt those changes creeping in.

I have been experimenting with a **growth hormone secretagogue peptide protocol** intended to improve some of these age-related changes. These peptides don't replace growth hormone the way synthetic HGH does. Instead, they stimulate the pituitary to release its own GH, potentially increasing the amplitude of GH pulses while retaining aspects of the body's natural regulatory mechanisms. [1-4]

The results were dramatic, and I learned a lot. Then, the washout period taught me even more about how changes in GH signaling might affect daily life.

Disclaimer: These notes are not authoritative and should not be interpreted as definitive scientific guidance. They are summaries, compilations and plausible extrapolations drawn from the most relevant research I was able to locate, and may omit nuances, conflicting data, or emerging evidence. The material reflects an attempt to organize and understand complex topics, not to provide clinical recommendations or expert interpretation.

Lifestyle Foundation

Peptides amplify lifestyle, not replace it. I maintain heavy resistance training twice weekly, aerobic conditioning twice weekly, a nutrient-dense plant-based diet, and a consistent sleep schedule. I believe this foundation helped translate changes in GH signaling into functional improvements.

Before I started GH peptides, I was already at goal weight and had been for more than a year. GLP-1-based treatment helped me obtain and maintain that goal. I continue retatrutide for metabolic support and appetite control.

Even with more than a year at goal weight and regular resistance training, I felt my body composition could be improved.

What are GH Secretagogue Peptides?

Growth hormone secretagogues encourage the pituitary gland to release its own growth hormone rather than supplying GH directly.

Common GH Secretagogue peptides operate through two related mechanisms:

- **Tesamorelin and CJC-1295 NODAC** act through growth hormone-releasing hormone (GHRH) receptors.

- **Ipamorelin** acts through the ghrelin/growth hormone secretagogue receptor (GHS-R1a).

Both approaches stimulate endogenous GH secretion. Human studies establish GH-stimulating activity for tesamorelin, ipamorelin, and long-acting CJC-1295, although the commonly marketed CJC-1295 NODAC formulation is distinct from the long-acting compound evaluated in the published CJC-1295 trials. [3,4,7,8]

Rather than supplying GH directly, these compounds stimulate the body's existing GH-production system. The resulting secretion remains subject to physiological feedback, although treatment can change the natural pattern and magnitude of GH exposure.

Why I Prefer GH Secretagogues Over Synthetic HGH

First, a quick biology lesson.

Your pituitary releases growth hormone in **roughly a dozen or more pulses throughout the day**, with substantial variation between individuals. In men, the largest and most reproducible pulse typically occurs shortly after sleep onset, during the first episode of deep slow-wave sleep. GH secretion and pulse amplitude decline substantially with age. [1,2]

These pulses are followed by periods of much lower GH concentrations. That pulsatile pattern appears to matter for some downstream metabolic effects.

One particularly interesting distinction is lipolysis, the mobilization of stored fat.

In a controlled human experiment, Cersosimo and colleagues demonstrated that pulsatile GH administration increased fatty-acid mobilization, while continuous administration of the same total GH dose did not produce the same response. [5]

A second human study comparing pulsatile and continuous GH administration found that pulsatile delivery produced greater lipolysis, while continuous delivery generated greater IGF-1 responses. Both delivery patterns impaired insulin sensitivity to a similar degree. [6]

This suggests an important principle: **the pattern of GH exposure can influence which downstream effects are emphasized.**

Synthetic HGH injections also produce a rise and fall in GH concentrations; they are not equivalent to a continuous infusion. However, GH secretagogues offer a different approach by stimulating endogenous secretion through the pituitary.

I find that approach mechanistically attractive, particularly for attempting to augment natural GH pulses associated with sleep, fasting, and lipolysis.

This document provides educational summaries of limited and evolving peptide data. Nothing here is medical advice for diagnosis or treatment. © 2026 Researcher6076

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My mechanistic preferences does not establish that GH secretagogues are inherently safer than synthetic HGH. Both approaches can increase IGF-1, produce fluid retention, and adversely affect glucose regulation. [1,10]

Human biology is adapted to GH arriving in pulses, and I am interested in working with that physiology rather than simply maximizing total GH exposure.

Who Might Benefit from GH Secretagogues?

The potential applications that interest me most are body composition, recovery, strength, and physical function in active older adults.

There is supporting human evidence, although outcomes vary by compound and population.

In a randomized trial of 412 adults with HIV-associated abdominal fat accumulation, tesamorelin reduced visceral adipose tissue by approximately 15% over 26 weeks, compared with an increase of approximately 5% in the placebo group. IGF-1 increased substantially. [7]

A separate randomized trial in 60 abdominally obese adults with reduced GH secretion found that 12 months of tesamorelin reduced visceral fat and improved several metabolic and cardiovascular risk markers. [8]

There is also relevant evidence involving GH secretagogues in older adults. A randomized study of 395 adults aged 65–84 evaluated capromorelin, an oral GH secretagogue. Treatment increased lean body mass and produced modest improvements in tandem walking and stair-climbing performance. [9]

GH/IGF-1 signaling has demonstrated effects on connective-tissue metabolism. A controlled human study found that GH administration increased tendon collagen synthesis, providing mechanistic support for a possible role in tissue remodeling and repair. [11]

Together, these results suggest that stimulating endogenous GH may produce meaningful changes in body composition and some measures of physical function.

However, increases in lean mass do not necessarily translate into proportional increases in muscle strength. A systematic review of **synthetic GH** treatment in healthy older adults found improvements in body composition but more adverse effects and insufficient evidence of broad functional benefits. [10]

For my purposes, the attraction is the possibility of improving recovery and body composition alongside an existing exercise and nutrition program.

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My 12-Week GH Peptide Protocol

My first 12-week cycle was:

- **Evenings**, fasted and immediately before bed: **Ipamorelin 200 mcg** paired with **CJC-1295 noDAC 200 mcg**.
- **Mornings**, fasted upon waking: **Tesamorelin 2 mg** along with **Ipamorelin 200 mcg**, followed by a three-hour fasting window.
- **Cycling:** 12 weeks ON, followed by 6 weeks OFF.

My current 12-week GH-axis cycle is as follows:

- **Evenings**, fasted and immediately before bed, **Tesamorelin 1 mg**. I bring the injection to bed and administer it immediately before turning out the lights, attempting to augment the first major sleep-associated GH pulse. This may plausibly support overnight tissue repair and recovery.
- **Mornings**, fasting for three hours afterward, **Tesamorelin 1 mg**. I prepare the injection in advance so I can administer it immediately upon waking. The goal is to stimulate morning GH secretion and its lipolytic effects, potentially promoting fat utilization during the subsequent fasting period. Human studies support GH-mediated lipolysis, although this specific morning timing and fasting schedule have not been tested for superior fat loss. [5,6]
- **Cycling:** After 12 weeks ON, cycle OFF for 6 weeks, or discontinue earlier if IGF-1 becomes excessively elevated or significant adverse effects develop.

The mechanistic rationale for splitting my daily 2 mg tesamorelin into two 1 mg doses is discussed here.

This divided-dose approach is experimental. The published body-composition trials generally used 2 mg once daily of the original tesamorelin formulation. Whether two 1 mg doses produce equivalent or superior clinical effects is not established. [7,8,14]

Functional Improvements I Experienced

GH secretion and circulating IGF-1 generally decline with age. Clinical trials show that pharmacological stimulation of endogenous GH can substantially increase IGF-1, sometimes reaching concentrations commonly observed in younger adults. [1,7,8]

This is an interesting biochemical comparison, but an increase in IGF-1 is not equivalent to reversing biological age.

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My most meaningful improvements, however, were not abstract biomarker shifts. They were daily, felt, and unmistakable.

Over the 12 weeks, recovery accelerated noticeably. **Muscle soreness diminished, recovery between workouts felt faster**, and back-to-back training days became easier.

Strength and functional capacity improved, making stair climbing, lifting, carrying, and balance tasks feel more natural and easier. My daily energy stabilized, afternoon fatigue diminished, and **stamina for long days increased**.

Body composition changed in ways **others noticed without prompting**. **My waist and neck leaned out**, and muscle retention appeared to improve.

Sleep deepened, awakenings decreased, and morning alertness sharpened. Tissue healing also seemed faster, with quicker recovery from minor injuries and joint strain.

Some of these observations are biologically plausible. GH/IGF-1 signaling contributes to connective-tissue metabolism, body composition, and physiological responses to training. [8,9,11]

These were not subtle changes; they were meaningful improvements in my daily life.

12 Weeks On, 6 Weeks Off

My decision to cycle GH peptides is based on a mechanistic hypothesis: **periodic washout may permit GH-axis signaling and downstream physiological systems to return toward their untreated state**, potentially limiting adaptation to sustained pharmacological stimulation.

Possible adaptations include changes in feedback regulation, receptor responsiveness, and downstream signaling. These mechanisms are biologically plausible, but it has not been demonstrated that a 12-week ON / 6-week OFF schedule prevents them.

Antibody formation is a separate consideration. Clinical studies have documented anti-tesamorelin antibodies, including neutralizing antibodies in a minority of treated patients. However, there is no established evidence that six-week washouts prevent antibody development. [14]

During washout, the direct stimulation of GH secretion ends, and treatment-related IGF-1 elevations would generally be expected to decline toward baseline.

Some changes are less likely to persist. In clinical studies, tesamorelin-related reductions in visceral fat were substantially lost after treatment discontinuation. [12]

On the other hand, improvements resulting from exercise, such as acquired strength, conditioning, and motor learning, may persist if training continues.

My hypothesis is that the peptides can help accelerate progress during the ON cycle, while continued training and nutrition preserve many of those gains during the OFF cycle.

My first washout offered an opportunity to test that idea against my own experience.

My Washout Experience

During the first two weeks of washout, my sleep was lighter, more fragmented, and less restorative.

By week three, sleep improved. This suggested to me that some physiological adaptation might have occurred during the preceding 12-week ON cycle, although changes in GH/IGF-1 signaling, sleep regulation, or unrelated factors could also explain the pattern.

Recovery slowed noticeably. Gym sessions felt harder, soreness lingered longer, and the **extra gear** I had during the protocol softened.

My interpretation was that augmented GH signaling might have been contributing meaningfully to my recovery during training.

Although I had not noticed the gradual increase in my appetite during my 12 weeks on GH peptides, I noticed that my appetite dropped dramatically during the OFF cycle. I reduced retatrutide to balance my declining appetite during washout.

The relationship between GH-axis stimulation and appetite is complicated, particularly when a GLP-1/GIP/glucagon receptor agonist such as retatrutide is also involved. I will watch this carefully when I restart GH secretagogues.

One surprising change was my resting heart rate. During the CJC-1295 noDAC/Ipamorelin/Tesamorelin GH cycle, my overnight resting heart rate averaged approximately **6–8 bpm higher**, an increase that was statistically significant in my own analysis.

By week two of washout, my resting heart rate returned to its previous baseline of approximately **52 bpm**.

I had originally attributed the higher RHR to retatrutide, which is known to produce dose-dependent increases in heart rate in human trials. [13]

The substantial decline during GH-peptide washout, however, suggests that GH-axis stimulation may also have contributed. That interpretation remains uncertain because the

observation was not from a controlled experiment and other factors could have changed simultaneously. I intend to track heart rate carefully during my current cycle, which uses a different pattern of GH secretagogues.

Despite these changes, I retained much of the strength, conditioning, and visible body-composition improvement I had developed during the cycle.

The experience was consistent with my hypothesis: the peptides may have accelerated progress, but at least some of the progress remained.

I Will Continue Cycling GH Peptides

Given the functional improvements I experienced, retention of much of my training progress, manageable washout, and strong lifestyle foundation, I intend to repeat my **12 weeks ON / 6 weeks OFF** schedule.

The schedule remains an individual experiment rather than an evidence-based cycling protocol.

My goals for the next cycle are to monitor IGF-1, glucose regulation, sleep, resting heart rate, training recovery, appetite, and functional performance more systematically.

I want to determine whether the changes in sleep, appetite, and resting heart rate recur during another ON/OFF transition.

Monitoring

IGF-1 monitoring is an important guardrail, but it is not the only one.

Tesamorelin stimulates endogenous GH secretion and can substantially increase circulating IGF-1. Clinical trials documented elevations above age-adjusted reference ranges in a significant proportion of treated patients. [7,14]

IGF-1 measurements help identify excessive GH-axis stimulation. However, a value within the age-adjusted reference range does not by itself establish that a protocol is safe.

The tesamorelin prescribing information recommends monitoring IGF-1 and considering discontinuation with persistent elevations, particularly above **3 standard deviation scores (SDS)**. It also identifies potential adverse effects including glucose intolerance, fluid retention, joint pain, and hypersensitivity reactions. [14]

My monitoring priorities include **IGF-1, fasting glucose, HbA1c, resting heart rate, blood pressure, fluid retention, sleep quality, and symptoms suggesting excessive GH activity**.

Particularly elevated IGF-1, deteriorating glucose regulation, or meaningful adverse effects would warrant reassessing or discontinuing treatment rather than simply relying on the scheduled washout.

Final Thoughts

I know GH secretagogues aren't magic, but my personal experience suggests they can be a useful tool for **recovery, strength, energy, sleep, body composition, and daily vitality**. There is human evidence supporting GH-axis effects on visceral fat, lean mass, collagen metabolism, and selected measures of physical function. [7-11]

The broader proposition that these particular peptides reliably improve all those outcomes in healthy, active older adults remains an extrapolation. For someone already committed to heavy training, good nutrition, and consistent sleep, I find the possibility of improving physiological recovery particularly compelling.

My first cycle felt like turning the clock back years. The washout provided a useful reminder that some improvements may be temporary, while others can persist through continued training.

I will continue experimenting, measuring the outcomes that I can, and refining the protocol based on what I observe.

It was remarkable to see real body recomposition happening over just a few weeks.

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